

--SUMMARY--

Decision No. 946/12

04-Nov-2014

S.Martel

- Fatigue
- Significant contribution (of compensable accident to subsequent disability)

The worker slipped and fell in April 2003, suffering minor organic injuries. Subsequently, she was diagnosed with chronic fatigue syndrome. The worker appealed a decision of the Appeals Resolution Officer denying ongoing entitlement.

The worker had a very significant pre-accident history, including active treatment. She suffered a number of pre-accident head injuries, with suspected concussions. The compensable accident did not result in any significant organic injuries. There were a number of non-compensable events subsequent to the accident that resulted in similar symptoms. The Vice-Chair concluded that the workplace accident did not significantly contribute to the worker's disability and loss of earnings beyond January 2004. The worker was entitled to ongoing LOE benefits until January 2004 only. The appeal was allowed in part.

18 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w5014s]
- TRIBUNAL DECISIONS CONSIDERED: 2697/08 refd to, 1158/10 refd to

Style of Cause:

Neutral Citation: 2014 ONWSIAT 2364



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 946/12

BEFORE: S. Martel: Vice-Chair

HEARING: May 14, 2012, November 14, 2013 and March 20, 2014 at Toronto
Oral

DATE OF DECISION: November 4, 2014

NEUTRAL CITATION: 2014 ONWSIAT 2364

DECISION UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated
February 27, 2009

APPEARANCES:

For the worker: S. Turkington, Lawyer

For the employer: T. McDonald, Paralegal

Interpreter: N/A

REASONS

(i) Introduction

- [1] The worker appeals the ARO decision of February 27, 2009, which denied entitlement to chronic fatigue syndrome (CFS) and post-concussive syndrome as related to an accident that occurred on April 7, 2003 when the worker slipped and fell on ice in a parking lot.
- [2] In this appeal, the worker seeks entitlement for ongoing symptoms and entitlement to loss of earnings (LOE) benefits after April 24, 2003.
- [3] The worker testified and the representatives made submissions. I also considered 14 documents entered as exhibits.

(ii) Background

- [4] The now 55-year-old teacher began working for the accident employer, a school board, as a high school teacher in 1991. On April 7, 2003, she slipped and fell on ice in the school parking lot while walking to her car. She fell backwards on her buttocks and injured her right hand and left elbow. She also reported that her head “snapped back.” She reported the incident the following day.
- [5] The worker saw an athletic therapist on April 9, 2003 and her family physician, Dr. Pasternak, on April 11, 2003, who diagnosed a whiplash and concussion. The worker stopped working on April 10, 2003.
- [6] The worker claimed ongoing symptoms after her fall and the Board referred her to the Toronto Rehabilitation Institute’s (TRI) neurology services for full assessment to clarify the diagnosis, prognosis and return to work. Upon discharge, the diagnoses were “possible mild concussion, no brain injury, resolved”, “cervical strain, resolved”, “tension headaches, accident related.” While the worker had subjective complaints of cognitive problems, on testing and examination, there were variable performance on measures of attention with average to superior performance on all of her sample domains and it was felt that cognitive problems were likely related to emotional distress.
- [7] The Board held that the worker's inability to return to work was not related to the fall of April 7, 2003 and limited LOE benefits to April 24, 2003. The Board held that the mechanics of the fall would not cause a concussive syndrome. The worker returned to work in the new school year in September 2003 but on a part-time basis. Since 2003, the worker has generally worked part-time with some unsuccessful attempts at full-time work as well as some periods of complete inability to work. The worker has been diagnosed with CFS.

(iii) Testimony

- [8] The worker provided testimony about her pre- and post-accident history.
- [9] Regarding her pre-accident history, she testified that she underwent knee surgery in the summer of 1990 and made a full recovery. She testified that she hurt her low back while working as a mother’s helper when she was a teenager. She also injured her sacroiliac joints the first year of teaching and has had periodic problems with her sacroiliac joints. The worker agreed that she suffered from stress approximately in 1998 or 1999 as a result of multiple stressors including her mother’s breast cancer diagnosis and volunteer work she was doing. She

stopped teaching in the spring and was able to return to work the following school year in September. The worker also suffers from asthma, which is controlled, as well as low blood pressure but does not require medication for the low blood pressure.

[10] Prior to 2001, the worker testified that she saw a chiropractor for one to two years and also saw an osteopath on a somewhat regular basis.

[11] The worker was involved in a motor vehicle accident in August 2001. She testified that she was the third car stopped at a light and as the light turned green, the cars ahead of her started to go but then suddenly stopped and the worker was rear-ended by the person behind her who did not stop in time. The worker estimated that the speed involved in the motor vehicle accident of August 2001 was only 5 to 10 kilometers per hour. The worker did not attend the hospital but was advised to see an athletic therapist by her massage therapist. The worker testified that the therapist treated her for her accident, muscle imbalance and for her low back. She testified that she was given strengthening exercises for her upper body and neck and that she was being weaned off the treatment prior to her fall of April 7, 2003. She testified that while the initial treatment was as a result of the motor vehicle accident, the ongoing treatment was for overall health improvements. The worker testified that other than a day here and there, she did not lose time from work but she did cancel a summer conference. She denied any cognitive issues arising from the motor vehicle accident.

[12] The worker testified that prior to her work injury, she was prescribed Elavil to help with her sleep as a result of disrupted sleep due to nerve issues in her toes. The worker denied that the Elavil was prescribed for depression. She testified that she was sleep deprived and not depressed.

[13] The worker testified that she generally had an allowance of 20 sick days per school year and that prior to 2003, she generally took 12 to 13 days of sick time as shown on the employer's attendance records.

[14] The worker testified that on April 7, 2003, she was walking to her car at the end of the school day when she slipped on ice. Both feet went up in the air and she fell to the ground. She was afraid that she had broken something. She felt pain in her right wrist because she fell on it in order to break the fall. She also experienced pain in the left elbow, back, neck and head. She returned inside the school and was subsequently assisted by a colleague back to her car and she drove home. That evening, she experienced nausea and a headache, which she treated with Tylenol. She went to work the following two days but was in a great deal of pain. She saw her athletic therapist and her family physician, who both recommended that she take some time off work. The worker testified that she had nausea associated with migraine-type headaches and that it was taking her longer to do tasks associated with teaching. She was unable to continue working out at the gym. She was off work for the remainder of the school year as well as the summer and while some of her symptoms improved with rest, she had ongoing headaches, fatigue and nausea. She also noticed that she began omitting letters in words.

[15] The worker agreed that she had difficulties with the MRI she underwent in July 2003. She found the noise very invasive and felt sedated when the MRI was done even though she had not been sedated.

[16] When she returned to work in September 2003, she only taught two instead of three classes. For the second semester, she only taught one class. She continued seeing the athletic

therapist, an osteopath and a massage therapist. In January 2004, she hit the top of her head on the open hatch door of her car. She had a significant headache with some nausea that lasted one week or more.

[17] In September 2004, the worker tried to return to work full time but found it physically taxing and her symptoms increased. She had memory difficulties. Her family physician recommended that she again decrease the number of classes taught. The worker also worked part time the following school year. The worker provided testimony regarding subsequent school years and the number and type of classes she taught each semester. In general, the worker was a very good historian.

[18] The worker testified that since the accident, she has had to stop extracurricular volunteer work, needlework and working out. If she does too much, she suffers from constant headaches and crushing fatigue.

[19] The worker also suffered subsequent motor vehicle accidents including one in April 2010 when someone backed out of a parking spot and hit the passenger back side of her car. She suffered from low back pain, neck pain and nausea, which subsided within approximately one month.

[20] The worker admitted that she had headaches prior to April 2003, but testified that these were related to a corn allergy. Once she stopped eating corn, her migraines ceased. Since April 2003, the worker has headaches with pain felt frontally, on top of the head and throughout her head. When severe, it is associated with nausea.

[21] The worker testified that her right hand and elbow pain resolved after the accident. The neck and head pain remain an issue and the back pain is fairly well resolved unless she reinjures it. She also has fatigue, which she noticed immediately after the accident and which persists. She has also experienced cognitive difficulties that worsen or improve depending on her pacing. She continues to undergo various therapeutic modalities such as physical therapy one to two times per week.

[22] The worker believes that her brain was damaged in the April 7, 2003 fall. She believes that her life changed significantly for the worse that day.

(iv) Documentary evidence

(a) Previous suspected concussions

[23] In a letter received by the Board on June 6, 2003, the worker provided a list of her four suspected concussions suffered prior to the work accident:

1. Sometime between 1980 and 1982 the worker fell backwards while skating and hit her head. She suffered a headache for one week with no other symptoms.
2. Sometime between September 1985 and June 1986, the worker was hit by a basketball in the face and almost lost consciousness. She suffered intermittent headaches, nausea and dizziness for about one week.
3. In the spring of 1991, the worker hit her head on the side edge of a car door and subsequently napped for a few hours. She suffered a headache for a few days.

4. On August 15, 1997, the worker hit the back side of her head on a doorknob and became dizzy and felt that she would lose consciousness. She napped for about three hours. She suffered from headaches, dizziness and nausea for about one week.

(b) Medical evidence

- [24] It appears that following the August 2001 motor vehicle accident, the worker saw Dr. Vokac, her then family physician, on September 16, 2001 and complained of occipital headaches. She saw Dr. Pasternak on October 18, 2001 and she was then taking Tylenol #3. While Dr. Pasternak's notes are not completely legible, it appears that the worker complained of headaches, almost daily, at the front and back along with nausea from which she was suffering that particular day.
- [25] The worker testified that she saw an osteopath from time to time prior to her work accident. A report dated October 31, 2004 from the osteopath states that she first assessed the worker on February 21, 2002, at which time her complaint was of "aggravated low back and neck pain producing headaches subsequent to a motor vehicle accident on August 25, 2001." The worker's previous history included periodic treatment for chronic sacroiliac joint pain and dysfunction, neck tightness and stiffness. The osteopath treated the worker for 15 sessions from March 2002 to December 2002 to return her to her pre-accident level of functioning. She then treated the worker on three occasions January 16, 23 and March 13, 2003 to address her chronic sacroiliac joint dysfunction. The worker did not return to see her until January 12, 2004.
- [26] As noted earlier, the worker sought treatment from an athletic therapist after the 2001 motor vehicle accident. According to the clinical notes of the athletic therapist, the worker saw him on a regular basis from October 2001 right up to the date of accident. Initially, when seen in the fall of 2001, the worker complained of headaches along with other complaints. Throughout the treatment prior to April 2003, it appears that most of the treatment concerned the "glut"¹ area but occasionally the worker complained of various exacerbations following various events. For example, on January 14, 2002, the worker reported that she had sneezed and irritated her low back. On June 10 and 12, 2002, she had irritated her lumbar pain as a result of coughing. On June 17, 2002, she reported that coughing had strained her neck. Generally, the worker attended the athletic therapist once or twice per week. In the months prior to the work-related accident, she saw the athletic therapist on February 3, 10, 19, 26, 2003, March 3, 6, 10, 17, 20, 26, 2003 and April 2, 2003. Contrary to the worker's testimony, I do not accept this frequency of treatment as being "weaned off" treatment.
- [27] Following the work accident, the worker saw the athletic therapist on April 8, 2003 and reported that she had fallen on ice and landed on her butt. In the following visits, she reported some symptoms of headaches, dizziness and nausea. By August 21, 2003, it was noted that the worker was biking longer. By September 2003, the frequency of her visits to the athletic therapist was similar to prior to April 2003. Furthermore, similar to prior to the work accident, many of the exercises were aimed towards the glut area. There are occasional references to periods of exacerbations following various events. For example, in October 2004, the worker reported a fall on her buttock while skating. The worker appears to have continued with ongoing treatment right up to the Tribunal hearing.

¹ Likely refers to "gluteus."

- [28] After the work accident, the worker saw Dr. Pasternak on April 11, 2003 complaining of headaches, back pain and neck pain. Dr. Pasternak provided a diagnosis of whiplash and concussion.
- [29] The worker was referred to Dr. Wong, a specialist in physical medicine and rehabilitation, who saw her on June 25, 2003. The worker reported being involved in a motor vehicle accident two years ago and was slowly noticing a gradual improvement in her endurance and pain control when she slipped at work on April 7, 2003. She landed on her buttock and did not strike her head. Since then, she was complaining of a low grade headache along the occipital region which was aggravated with physical activity. She had noticed reduced physical endurance. She had some problems with memory in terms of word spellings but this had since resolved unless she overexerted herself. Dr. Wong was unable to come up with a clear explanation for the worker's low endurance. The worker had symptoms suggestive of a cervicogenic type headache and he suggested a cortisone or Botox injection.
- [30] The Board referred the worker to the TRI for a complete assessment where the worker was assessed by multiple disciplines: physiatry, neurology, neuropsychology, psychiatry, occupational therapy and physiotherapy. The neuropsychologist noted that during the clinical interview, the worker offered an extremely detailed and at times overly descriptive account of her accident and subsequent difficulties. It was noted that she did not strike her head as a result of the accident. The results of the worker's neuropsychological assessment revealed variable performance on measures of attention, in the context of average to superior performance on all other sample domains. The neuropsychologist opined that the worker's subjective cognitive problems were likely related to the effects of emotional distress and her cognition. On the basis of the accident report, there was no evidence to suggest that she would be suffering from any cognitive post-concussive symptoms. As such, her cognitive status did not preclude a return to her previous duties and responsibilities. Given her pre-morbid history of anxiety and depression, they recommended that the transition occur on a gradual basis to minimize any potential adjustment difficulties. At the time of her evaluation, her reported emotional symptoms were mild and did not meet full criteria for a mood disorder diagnosis. There did not appear to be any emotional contraindication to her returning to work. In respect of the pre-accident history, the neuropsychologist noted that the worker suffered a period of depression in 1987 following a move to Alberta. She saw a psychologist for approximately one year for supportive counselling. Since that time, she had also participated in alternative therapy for depression.
- [31] Dr. Van Reekum, a psychiatrist, provided a report on July 3, 2003 stating that the worker was on Elavil, which she had started after the accident, but which was given to her for a problem that started about a month or two prior to the accident, in respect of a nerve firing in her left big toe, which affected her sleep. The worker indicated that she had seen a psychiatrist and psychologist in the past but had not taken antidepressants. Four years ago, she was under a lot of stress related to a difficult time, some volunteer work, an abnormal pap test and her mother developing breast cancer. Since then, she had been told by her family physician that she was depressed. She found that her mood tended to be worse in the winter. On examination, there were no psychiatric signs. In summary, Dr. Van Reekum indicated that while the worker had been told that she was suffering from depression in the past, it did not appear that her depressive symptoms would have met the threshold for major depressive episode but this was difficult to assess retrospectively. It did not appear that she had a formal Axis I psychiatric diagnosis at

present. Since the accident, she had developed pain, which was causing insomnia and contributing to subjective cognitive complaints.

[32] Dr. Watson, a neurologist, provided a report on July 8, 2003 opining that there was no significant neurological injury, disease, disorder, impairment or disability that could be reasonably related to the injury. The worker's headaches were tension type and should respond best to measures aimed at muscular pain.

[33] The TRI conference summary report of August 7, 2003, upon discharge, noted that an MRI had revealed no evidence of brain trauma. The diagnoses were:

1. Possible mild concussion, no brain injury, resolved.
2. Cervical strain, resolved.
3. Tension headaches, accident related.
4. No Axis I psychiatric diagnoses.

[34] A number of recommendations were made in respect of medication and non-pharmacological measures. A gradual return to work and duties over four weeks was recommended as well.

[35] Dr. Wong reassessed the worker on August 7, 2003. The worker apparently advised Dr. Wong that she had had psychometric testing and the results suggested a mild concussion; however, the MRI of the brain was negative. Dr. Wong felt that the worker's symptoms suggested a mild concussion. There were no cognitive deficits other than extreme fatigue following any strenuous type of activity whether physical or cognitive. He reassured the worker that there would be continual slow improvement, which she had noticed. He suggested Botox injections for the headaches.

[36] On May 4, 2004, Dr. Wong reported that the worker was teary when she informed him that she had just been turned down for long-term disability. Dr. Wong reported that unfortunately, her symptoms suggestive of only mild post-concussion syndrome would make it difficult for her to qualify for this. Her main symptom was fatigue and he started her on Alerte to try and boost her energy. Dr. Wong's notes indicate that in July 2004, the worker complained of pain, nausea, severe fatigue and weight loss following a gum graft and that she rested through July and August.

[37] Dr. Pasternak provided a report on October 13, 2004 regarding the worker's injuries and symptoms following the fall at work. He reported that when he saw the worker on April 11, 2003, he concluded that the worker had suffered a flexion extension injury to her neck, upper back and a concussion. On July 21, 2003, the worker had an MRI exam and the next day was very bad "the worse day since the accident." She had increased headaches and nausea. In September 2003, she was working half days and felt extreme fatigue and poor concentration and nausea. When seen on January 17, 2004, she had hit her head on the hatch of her car resulting in increased headache and nausea. He diagnosed a concussion and advised her to rest. In August 2004, the worker was tired, having had gum surgery. In conclusion, Dr. Pasternak indicated that the worker apparently had no prior history of head trauma and was experiencing recognized symptoms of concussion after her fall of 2003. Her subsequent course of headaches, nausea, concentration problems, memory deficit, anxiety, and fatigue were characteristic of a persistent post-concussion syndrome. Dr. Pasternak also noted that he had seen the worker on October 18, 2001 for injuries received in a motor vehicle accident, which occurred

August 25, 2001. She had whiplash related neck and head pain and she recovered to her pre-accident status with treatment within a few months.

[38] Dr. Wong provided a report on February 28, 2005 addressed to the worker's counsel in respect of her disability income claim. He noted that the worker's overriding complaint was fatigue and cognitive deficits. She met the criteria for a chronic fatigue syndrome as she was experiencing her symptoms for greater than six months in the absence of any other medical condition. He suspected that this was the direct result of her initial slip and fall at work in which she sustained a flexion and extension type injury. The mechanism of injury was similar to an individual involved in a motor vehicle accident. Dr. Wong reported that direct trauma to the brain or skull is not necessary for a diagnosis of a mild concussive syndrome as the rapid acceleration and deceleration can cause injury to the axons. It is not clear why individuals develop chronic fatigue syndrome. In many instances it is idiopathic whereas in other instances it may follow chronic conditions or injuries such as the worker sustained. He noted that he did not know what the worker's neuropsychological scores may have been prior to her injury but her current psychometric testing revealed that she did not have any significant cognitive deficits. Dr. Wong was unable to comment on whether the worker's persistent symptoms were the result of emotional distress because this was outside his area of expertise. He believed that the worker was disabled from working full time.

[39] The worker's counsel for her disability claim referred her to Dr. Mossanen, a neurologist, for an independent neurological and pain evaluation, done in March 2005. Dr. Mossanen noted the worker's previous history of head traumas but indicated that since none of them resulted in loss of consciousness, the term concussion could not be applied to them. In respect of the fall at school, the worker indicated that she remembered that her head was severely thrown forward and backwards twice. Dr. Mossanen indicated that he did not believe that the worker sustained any concussion because by definition, concussion is defined as a temporary loss of consciousness secondary to axonal disruption. The worker sustained whiplash injury and post-traumatic headaches, nausea and disequilibrium are not uncommon to injuries such as these, but these had now all resolved. He did not believe that the worker sustained a traumatic brain injury that can be objectively shown by present diagnostic tests. He noted, however, that difficulty with physical and mental endurance is not unusual in traumatic injuries such as this even without a loss of consciousness. There was nothing in the worker's history that suggested that she was fabricating her story or malingering. The fact that the physical examination, neuropsychological assessment and MRI examination showed no abnormality did not necessarily mean that her symptoms, albeit subjective, were not genuine. If she had CFS, which was possible, her symptoms were not uncommonly associated with cognitive problems. Recent literature suggested that in some individuals who are predisposed to immune system activation of coagulation, trauma can trigger circulation disturbance. In summary, he believed that the worker's symptoms were genuine. He did not believe, however, that her symptoms were due to post-concussion syndrome. He felt that the worker's inability to increase her workload at school to the same level as prior to the accident had been compromised and that she was unable to undertake a full-time occupation. She was therefore considered partially disabled.

[40] Dr. Mossanen provided an addendum to his report, which is undated, but is stamped as having been received on July 11, 2005. He clarified that he felt that the worker was totally disabled with regard to participating in her teaching occupation on a full-time basis. He agreed that she had CFS. With respect to the issue of a biochemical basis, he mentioned that at least in

some circles, it is believed that trauma will trigger in a pre-disposed individual slowing down of the circulation not only to the musculature but also to the nervous system. He acknowledged that this concept is fairly new and to his knowledge, proper studies on a scientific basis had not yet been carried out.

[41] Dr. Pasternak also referred the worker to Dr. Tator, a neurosurgeon, who saw her on May 24, 2006. The worker reported a history of her feet going out from under her and falling backwards and while she did not hit her head, her head whiplashed twice. She estimated that she had had prior head injuries, none of which caused unconsciousness. She also admitted that she had some pre-existing depression prior to the injury with further depression since the injury because she wanted her life back. On examination, there were no neurological deficits. Superficial testing of her cognitive function revealed no major deficits. He reviewed the MRI, which was essentially normal. He opined that the worker was suffering from post-concussion syndrome and had several symptoms residual to this. He encouraged regular exercise and non-impact type of exercise.

[42] Dr. Bested, a haematological pathologist, provided a report to the worker's representative on August 24, 2007. Dr. Bested was of the view that the worker had severe CFS. She had severe role limitation as a result of her fatigue. It was her opinion that the accident of April 2003 was the contributing factor to the worker's situation of fibromyalgia and chronic fatigue syndrome. Dr. Bested reported that "it is commonly found that as a result of a physical trauma that fibromyalgia occurs and also sometimes chronic fatigue does occur after physical trauma." With respect to the prior motor vehicle accident, Dr. Bested indicated that it resulted in a whiplash injury without concussion. She had been referred to physiotherapy and had improved to the point where she was reducing therapy and attending every two to three weeks. She did not suffer from cognitive impairment as a result of that injury. Dr. Bested opined that the worker had recovered from the physical trauma from the 2001 accident from the history given.

[43] The employer obtained medical opinions from Cira Medical Services and in particular from four different medical specialties. Dr. Roussev, a neurologist, provided his file review on October 7, 2013. He opined that the probable diagnosis and the worker's recovery to her pre-accident state was as follows:

6. Having regard for all the available information, what is the probable diagnosis for the injuries sustained in the fall at work on April 7, 2003? Please explain.

The documentation presented indicates a slip and fall incident with the claimant landing on her buttocks, outstretched right hand, left elbow. There is no indication that the claimant has sustained any form of brain injury. Based on post incident symptoms, clinicians have offered a diagnosis of concussion, with a consensus of at most a mild concussion. Neurology evaluators such as Dr. Mossanen and Dr. Watson did not find evidence for brain injury.

By neurotrauma criteria, it is the opinion of this assessor that the claimant did not sustained [sic] any form of significant neurotrauma in the index [sic] accident of April 7, 2003. A diagnosis of concussion or cerebral injury is not substantiated. Myoligamentous strain would have been expected to have occurred and I would defer comment in this regard to Dr. R. Zabieliuskas, Physiatrist.

7. Having regard for the detailed medical reporting in the clinical notes, did the employee at any time recover to her pre-accident state, subsequent to the work injury? Please explain.

According to the documentation provided, predominantly by Dr. Wong, and the description of treatments and their results, in the early post incident period, there appears to have been success in managing the claimant's symptoms with a variety of interventions including Botox, Ritalin and antidepressants.

The treatment by Dr. Wong also shows a late escalation of symptoms circa 2008 and onwards. This is not explicable on the basis of any form of trauma sustained in the 2003 accident, from a neurological perspective. The persistence of symptoms for that length of time and their subsequent escalation cannot be plausibly attributed to any physiological effect of the slip and fall accident of 2003.

[44] Dr. West, a clinical and neuropsychological consultant felt that from a neuropsychological perspective, there was no compelling evidence of a traumatic brain injury and associated neurophysiological impairment as a result of the 2003 slip and fall injury.

[45] Dr. Zabieliuskas, a physiatrist, provided a report on October 4, 2013 and opined as follows:

OPINIONS AND CONCLUSIONS

The high school teacher sustained no more than an uncomplicated slip and fall on ice on April 7, 2003 in which she would have had no more than a cervical strain WAD II, a buttock strain/contusion, a right wrist strain and contusion, as well as a left elbow contusion. The teacher was able to get up off the ground and go back to the school to find a colleague who escorted her back to her car and she was able to drive home in icy conditions from the school parking lot. This would further support the conclusion that, within all reasonable medical certainty and based on a review of the provided file documentation, the teacher sustained nothing more than uncomplicated soft tissue strain injuries and contusions that would have healed by the time the comprehensive assessments were completed at the Toronto Rehabilitation Institute in June and early July 2003.

The evolution of symptoms and the excessive allied health professional involvement for an uncomplicated slip and fall cannot be supported on any objective organic basis and leads me to conclude that, within all reasonable medical certainty, by the time the school year began in September 2003, the teacher would have been safe to resume the work duties at graduated hours as recommended by the Toronto Rehabilitation Institute team leading to a full return to full work duties by the end of 2003. The subjective symptom of fatigue cannot be in any way related to the uncomplicated soft tissue strains and contusions sustained on April 7, 2003 with respect to my specialty of physical medicine and rehabilitation.

Any extensive treatments that the teacher received after the summer of 2003 would not have been related to the slip and fall of April 7, 2003. There appears to have been an overwhelming response to a rather uncomplicated simple slip and fall that occurred on April 7, 2003. A number of well-meaning but misguided allied health professionals appear to have created a situation that was not supported by any objective evidence of any ongoing physical impairment or physical disability attributable to the slip and fall of April 7, 2003 by the time the extensive evaluations were conducted at the Toronto Rehabilitation Institute, ending with the normal physical examinations of Dr. Masanic, Physiatrist, and Dr. Watson, Neurologist, in addition to the lack of any psychiatric condition noted by Dr. Van Reekum or any evidence of a concussion by the Neuropsychologist Dr. Kemenoff, in the report of August 25, 2003.

[46] Finally, Dr. Gratzner, a psychiatrist provided his opinion on October 4, 2013. Dr. Gratzner thought that the worker meets the DSM-5 criteria for a somatic symptom disorder:

Pain disorder is not recognized in DSM-5; the diagnosis has merged with other diagnoses and become a somatic symptom disorder.

The diagnostic criteria are as follows:

- a) One or more somatic symptoms that are distressing or result in significant disruption of daily life.
- b) Excessive thoughts, feelings or behaviours related to somatic symptoms or associated health concerns as manifested by at least one of the following:
 - 1. Disproportionate persistent thoughts about the seriousness of one's symptoms.
 - 2. Persistence in high level of anxiety about health or symptoms.
 - 3. Excessive time and energy devoted to these symptoms or health concerns.
- c) Although any one somatic symptom may be continuously present, the state of being somatic is persistent (typically more than six months).

Again, based on the File Review, based on notes of different health providers including the psychology report I have already highlighted, this would be the diagnosis which I believe best applies to this employee. Indeed, I suspect that this diagnosis occurred before she had her injury in 2003. It is difficult to fully judge how much recovery she has had, if any, but I would speculate she meets criteria for this today. Thus, the 2003 accident, occurred in somebody with a certain amount of vulnerability, who already was very somatic, and her injuries simply exacerbated, from a psychiatric perspective, her pre-existing condition.

Unfortunately, like many such individuals, this employee has met health care providers who - based on the File Review - may have worsened her condition with new diagnoses and treatments. Chronic fatigue syndrome, in particular, is highly controversial, and the diagnosis made by a pathologist who fashions herself to be an expert in this area is concerning.

[47] With respect to the pre-morbid psychiatric history, Dr. Gratzner responded that the worker had a history of difficulties coping. She had a history of seeking help and receiving psychotherapy. She also had a history of somatic issues including headaches. There was also concern about fatigue and the suggestion that she may have had chronic fatigue prior to 2003. He saw the worker as having some baseline neurosis and difficulties coping and suggested this was consistent with somatic symptom disorder. With respect to the incidents that occurred after April 2003, including the motor vehicle accidents, Dr. Gratzner thought that given the worker's difficulties coping with different stresses, both physical and emotional, further physical insults, such as motor vehicle accidents, would be difficult for her to tolerate. He considered this very significant in looking at the worker's overall recovery or inability to fully recover. He felt that the motor vehicle accidents were very relevant in the worker's difficulties coping and recovery.

(v) Submissions

(a) Worker's submissions

[48] Ms. Turkington submitted that the workplace accident of April 2003 substantially changed the worker's life for the worse and that as a result of that accident she was unable to return to work full time. The worker seeks LOE benefits for the lost wages. Ms. Turkington submitted that the workplace accident significantly contributed to the worker's disability.

[49] With respect to the worker's pre-accident history, Ms. Turkington submitted that there was some evidence that the worker lost time from work five years prior to the accident when her mother was diagnosed with breast cancer and that the worker's family physician then referenced depression. The symptoms, however, were different than the symptoms that she suffered post-accident in that she was not fatigued. In any event, there was a brief period of absence from

work and the worker was able to return to work full time in September. Ms. Turkington submitted that there was no other significant absence from work until the workplace accident of April 2003. Mr. Turkington submitted that the worker's low back condition prior to the accident was not significant since the worker had been able to continue working with no lost time. With respect to the prior head injuries, these were followed by only a brief period of symptoms and the worker was able to continue working full time. With respect to the 2001 motor vehicle accident, Ms. Turkington submitted that the worker's symptoms were largely resolved and that the records did not indicate any ongoing symptoms immediately prior to the workplace accident.

- [50] Ms. Turkington submitted that the medical picture after April 2003 was markedly different than prior to April 2003. The worker was unable to recover from this incident. She suffered from dizziness, headaches and neck pain. She was authorized time off work. The worker was assessed at the TRI and even at that time, four months after the accident, she was still experiencing ongoing headaches. The diagnosis was a possible mild concussion, headaches and cognitive difficulties related to emotional distress. While she was authorized to return to work, she was advised to take it easy.
- [51] The worker returned to work but was unable to return on a full-time basis. Dr. Wong's reports noted ongoing headaches and fatigue. The worker went on to see multiple physicians but they all reported similar ongoing symptoms consisting of headaches and fatigue. All of her physicians recommended that she reduce her teaching load. None of the physicians ever suggested that she had recovered. Physicians, such as Dr. Mossanen, did not question the worker's genuineness and opined that her symptoms were not unusual even following an accident with no loss of consciousness.
- [52] While there may have been subsequent personal issues, Ms. Turkington submitted that there was nothing significant after the workplace accident. The worker was naturally frustrated with her ongoing disability and was referred to various therapists. Dr. Tater provided a diagnosis of post-concussion syndrome and Dr. Wong and Dr. Bested provided diagnoses of CFS. In any event, the symptoms that the worker reported were consistent following the incident.
- [53] Even if one accepted the opinion of Dr. Gratzner that the worker had a somatic symptom disorder, Ms. Turkington submitted that the accident aggravated that disorder, an alternative basis for entitlement.
- [54] Ms. Turkington submitted that the medical information and Tribunal decisions indicate that a worker can suffer from post-concussion syndrome even in the absence of a head injury or head impact. She submitted that a whiplash type injury can lead to post-concussive syndrome. Regardless of the diagnosis, Ms. Turkington submitted that the worker's disability following the workplace accident was related to that accident.

(b) Employer submissions

- [55] Ms. McDonald submitted that the accident was not a significant contributing factor to the worker's condition. She agreed that an exact diagnosis was not necessary but submitted that the worker's symptoms did not all start with the workplace accident. Ms. McDonald referred to the worker's pre-accident history of depression, whiplash and head traumas. There was also reference to endurance and pain control problems prior to the accident. Even though the 2001 motor vehicle accident did not result in extensive lost time from work, there were significant ongoing symptoms that persisted for many months after the accident. Ms. McDonald submitted

that the pre-existing and co-existing conditions were significant contributing factors to the worker's persistent problems. She also noted the number of non-occupational incidents that post-dated the accident and for which the worker claimed symptoms. She submitted that by the summer/fall of 2003, the significance of the work accident was fading and was then overwhelmed by non-work-related factors. She submitted that the relative contribution of the work accident diminished with time. The worker's symptom complex pre-existed and persisted after the accident.

[56] Ms. McDonald submitted that there was no evidence of any brain damage despite the worker's belief that her brain was injured. She submitted that it was necessary to understand the exact mechanics of the injury and that the worker's physicians were not necessarily aware of everything other than the workplace accident. The worker's belief that the accident was the cause of her accident impacted on the history she provided to various medical practitioners.

[57] Ms. McDonald submitted that the worker's condition was primarily a non-organic symptom complex or a combined organic and non-organic symptom complex. She relied on the opinions of the Cira physicians, which she submitted provided a perspective based on the big picture taking into account the entire medical history. She submitted that the most probable explanation was the worker's difficulty coping with stress and her pre-existing somatic symptom disorder. She also relied on the TRI reports, which she submitted were comprehensive and based on a good understanding of the accident history. Ultimately, upon discharge, they provided a diagnosis of *possible* mild concussion, no brain injury, *resolved*. She submitted that there was no evidence of a post-concussion syndrome at that time. The worker's symptoms were essentially largely resolved at that time. Ms. McDonald agreed that it was reasonable to expect the worker to return to work in September 2003 on a gradual basis. The worker's main issue preventing a return to full-time work was fatigue, but Ms. McDonald noted that low endurance had been noted as a problem even prior to the accident. She also noted that according to Dr. Wong, the worker did not meet the criteria for fibromyalgia diagnosis.

[58] Ms. McDonald submitted that the worker's somatic symptoms waxed and waned with the events in her life such as physical falls, student difficulties and other emotional difficulties. It was unclear where Dr. Bested obtained the diagnosis of fibromyalgia. Ms. McDonald relied on *Decision No. 2697/08* in which an assessor opinion was preferred to that of Dr. Bested. She noted that if the worker's symptoms were in fact CFS then the same conclusion should be reached in this case as was reached in *Decision No. 2697/08* that that condition is not the result of trauma. Ultimately, Ms. McDonald submitted that while it was possible that the worker's recovery as a result of the accident lasted until January 2004, she submitted that there should be no entitlement beyond January 2004.

(vi) Conclusion

[59] After considering the evidence and submissions, I find that the workplace fall in April 2003 did not significantly contribute to the worker's ongoing complaints. I find that any compensable symptoms and lost time should have ended by the second semester of the 2003/2004 school year. My reasons follow.

[60] Firstly, I am of the view that there is a very significant pre-accident history, including active treatment at the time of the workplace accident. The worker suffered from a number of pre-accident head injuries. While none of them were associated with loss of consciousness, the

worker nevertheless experienced various symptoms of fatigue, dizziness and nausea lasting up to a week. The worker also has a history of seeking various treatment modalities on an ongoing basis rather than only during acute periods of injury. Prior to even the 2001 motor vehicle accident, the worker had a history of treatment from a chiropractor, osteopath and massage therapist. The TRI neuropsychologist also refers to ongoing alternative therapy for depression prior to the work accident.

[61] In August 2001, the worker was involved in a motor vehicle accident, which she described as involving low speeds of 5 to 10 kilometers per hour. Despite this very low speed accident, when the worker saw Dr. Pasternak on October 18, 2001, two months after the accident, she complained of nausea and daily headaches. She sought treatment from an athletic therapist following the motor vehicle accident. While I agree with the worker's testimony and Ms. Turkington's submission that not all of the treatment received by the athletic therapist was specifically directed to the motor vehicle accident injuries, I disagree that the worker was being "weaned" from this treatment prior to her workplace fall. On the contrary, immediately prior to the workplace fall, the worker continued to attend the athletic therapist at essentially the same frequency of once to twice per week. I also note Dr. Wong's report of June 25, 2003 relating the worker's report of slow improvement and gradual improvement in endurance and pain control following the motor vehicle accident and prior to the fall at work. This suggests that the worker had pain and endurance issues prior to the workplace fall, which were not all resolved prior to the fall at work. Finally, according to Dr. Van Reekum's report of July 3, 2003, the worker was then taking Elavil, which had been prescribed for a problem that started about a month or two prior to the workplace fall. The worker reported problems with a nerve firing in her toe, which affected her sleep. If the worker's sleep was disturbed to the point where she was prescribed medication to assist with sleeping, she was likely already fatigued prior to the workplace fall.

[62] Secondly, the workplace fall was not a significant accident nor did it result in any significant organic injuries. The worker's Form 6 completed on April 25, 2003, states that she fell backwards on her buttocks and injured her right hand and left elbow and that her head "snapped back." There is no indication on this form that the worker's head was severely thrown forward and backwards "twice," as is found in later medical reporting such as Dr. Mossanen's report of March 15, 2005 and Dr. Tator's report of May 24, 2006, which states that the worker's head whiplashed twice. There is also no question that the worker did not hit her head. She was subsequently able to drive herself home. While some physicians compare the whiplash injury to a motor vehicle accident, none have explained whether the speeds involved are even comparable. Ultimately, I prefer the most contemporaneous reporting regarding the accident history that there were injuries to the worker's right hand and left elbow as well as a single whiplash motion of the head. There is no evidence of any organic brain injury despite the worker's beliefs and concerns.

[63] I am also of the view that the most persuasive medical reporting regarding the nature of the worker's injuries as a result of the fall is the comprehensive TRI assessment. I rely on the TRI reporting because it was a multi-disciplinary assessment done just three months after the fall, therefore contemporaneous with the event. As part of that assessment, the worker was examined by Dr. Watson, a neurologist, who opined that there was no significant neurological injury, disease, disorder, impairment or disability that could be reasonably related to the injury. The cognitive testing done by the neuropsychologist did not reveal any objective deficits. On discharge, the worker was diagnosed with having suffered a "possible mild concussion, no brain injury, resolved." As submitted by Ms. McDonald, the assessors could not confirm a mild

concussion but even if the worker had suffered a mild concussion, it was resolved by the time she attended the TRI assessment. The only ongoing accident-related diagnosis was tension headaches. While a gradual rather than full return to work was recommended, this recommendation was as a result of the worker's pre-morbid history of anxiety and depression in order to minimize any potential adjustment difficulties. The gradual return to work was therefore not as a consequence of the nature of the injuries suffered in the fall but rather, with a view to accommodating the worker's pre-existing history in order to achieve a successful return to work.

[64] In addition to the TRI reports, there are other references in other reports to improvements in the worker's overall condition after the workplace fall. For example, when seen by Dr. Wong on June 25, 2003, the worker reported resolution of word spelling problems unless she overexerted herself. By August 21, 2003, the athletic therapist noted that the worker was biking longer.

[65] As noted by Ms. McDonald, a number of events after the workplace fall also occurred resulting in similar symptoms to those reported by the worker after her workplace fall. Some of these include:

1. According to Employee Assistance Program notes, from May to August 2003, the worker reported significant stress regarding an upcoming family reunion scheduled in August 2003.
2. The worker hit her head against the hatchback door of her car in January 2004, resulting in nausea and headaches. She was diagnosed as having suffered a concussion.
3. The worker reported symptoms of pain, nausea, severe fatigue and weight loss following gum graft surgery and rested through July and August 2004.
4. The worker reported relationship difficulties to her psychologist in June 2005, resulting in feelings of "numbness" and "detachment" from her body, similar to an anesthetic.
5. In August 2005, the worker reported to her psychologist that she was afraid of mammogram results and worried about not being able to cope with "one more thing."
6. The worker fell in November 2007 and suffered from nausea and headaches. She also reported feelings of depression about her repeated falls.
7. The worker suffered a motor vehicle accident in February 2010, resulting in a sore neck. She had another accident on April 9, 2010, resulting in headaches and a sore neck.

[66] Similar to the workplace fall of April 2003, none of the above events was particularly traumatic or severe but they all resulted in a number of physical and/or emotional symptoms, many of which were similar to the symptoms the worker attributes to the workplace fall. In pursuing her disability and worker's compensation claims, the worker has attributed increased attention and focus to the workplace fall without necessarily providing a complete pre-accident

and post-accident history of the course of her accidents and symptoms. In my view, the complete documentary evidence of significant reported subjective symptoms following the most minor of events, before and after the accident, supports a pre and post-accident diagnosis of somatic symptom disorder, as opined by Dr. Gratzner. The worker had a history of difficulties coping prior to the accident, had difficulty coping following the accident and difficulty coping following a number of other incidents and events that occurred after the workplace accident. The workplace accident was but one event amongst many in the ups and downs of the worker's condition.

[67] The worker's teaching history since September 2003 attests to these ups and downs where she is at times able to teach two classes, at times one and at times none. I cannot find, however, that the minor workplace fall significantly contributed to any permanent organic or non-organic symptoms. As indicated previously, I rely mostly on the TRI reports. By then, other than headaches, the worker's injuries resulting from the fall had resolved and a gradual return to work was recommended. There were no cognitive deficits. Given that the worker was a teacher at a semester school, I accept that the best way of ensuring a successful transitional return to work was to reduce her teaching load for the first semester. I therefore grant the worker LOE benefits for the time not worked until the end of January 2004. I cannot find that the accident significantly contributed to lost time after January 2004.

[68] Ms. Turkington submitted that the workplace fall was notable because previously the worker had been able to work full-time and afterwards was unable to do so. Firstly, I note that the workplace accident occurred during the school year as opposed to during the summer break, which explains the immediate lost time. There were also other post accident events, which similarly resulted in similar symptoms and reduction in the worker's work load. The worker hit her head against the hatchback door of her car in January 2004 resulting in nausea and headaches. She was diagnosed as having suffered a concussion. Whereas the worker had been teaching two classes during the first semester of the 2003 to 2004 school year, she reduced her class load to one class, the second semester, which started in February 2004, shortly after the January 2004 hatchback door incident. The medical reports also indicate that the worker experienced extreme fatigue following gum graft surgery that occurred in the summer of 2004, following which the worker required two months of rest. The worker attempted but was unable to return to teaching full time in the fall of 2004. She was then off work for a couple of months and when she returned to work, she only did so at a 50% teaching load. There were therefore multiple other incidents that resulted in a number of symptoms such as fatigue, nausea and headaches followed by an inability to teach a full course load. In the fall of 2009, the worker reported stress over school and the curriculum to the point where she was unable to work until the following school year. Whereas the worker had a 60% teaching load in September 2003, this was reduced to 40% around February 2004, zero shortly after the new school year in 2004, 50% in November 2004, 33% in December 2007 and an inability to work for most of the 2009-2010 and 2012-2013 school years. The further reductions in the worker's teaching load suggest that the 2003 fall did not significantly contribute to the worker's lost time after January 2004 but rather, that there has been a natural progression of the worker's somatic and fatigue complaints. In any event, a temporal connection on its own is not sufficient to establish causation between a minor incident and subsequent symptoms (see *Decision No. 1158/10*).

[69] With respect to the CFS diagnosis, I note this was first offered as a diagnosis by Dr. Wong on February 28, 2005, almost two years after the workplace fall. While some physicians have attributed this diagnosis to trauma, even Dr. Mossanen who accepted the

genuineness of the worker's symptoms, acknowledged that the concept of trauma triggering a slowing down of circulation is a fairly new concept. Proper scientific studies have not been carried out to support the concept. The assessor opinion quoted in *Decision No. 2697/08*, relied upon by the employer, similarly indicated that there is no literature supporting physical or psychological trauma as the trigger for CFS. It is therefore unclear how Dr. Bested can opine that "it is commonly found that as a result of a physical trauma that fibromyalgia occurs and also sometimes chronic fatigue does occur after physical trauma." Dr. Bested also diagnosed the worker with fibromyalgia without explaining the basis for this diagnosis. Dr. Wong had previously ruled out this diagnosis; the worker did not have the requisite tender points. Dr. Bested also has a mistaken view of the pre-accident history in that she states that the worker had reduced therapy to an attendance of every two to three weeks prior to the workplace accident, which is inaccurate. I therefore do not accept Dr. Bested's opinions.

[70] In summary, the worker had a significant pre-existing history and was in active treatment at the time of her workplace accident. The workplace accident was minor and did not result in any serious injuries. Other than headaches, the worker's compensable injuries had resolved by the time she underwent the TRI assessment. A gradual return to work was recommended to accommodate the pre-existing vulnerability. There were multiple other non-compensable incidents, starting with a diagnosed concussion in January 2004, which resulted in multiple symptoms and further lost time from work. I find that the workplace fall of April 2003 did not significantly contribute to the worker's disability and loss of earnings beyond January 2004.

DISPOSITION

- [71] The worker's appeal is allowed in part. The worker is entitled to ongoing LOE benefits for lost time incurred until the end of January 2004. There is no ongoing entitlement after January 2004.

DATED: November 4, 2014

SIGNED: S. Martel